Data Dictionary Codebook

HCP Competency Assessment (PID: 58462)

09/01/2025 9:26am

Instruments	
Instrument	**Form Name**
Hcp Competency Assessment	hcp_competency_assessment
Part Ii Questions For Screening And Counseling Sta	part_ii_questions_for_screening_and_counseling_sta
Form 1	form_1

Languages		
ID		**Display Name**
en	☑	English **(default)**
vi	☐	Vietnamese

#	Variable / Field Name	Field Label *Field Note*	Field Attributes (Field Type, Validation, Choices, Calculations, etc.)
Instrument: Hcp Competency Assessment (hcp_competency_assessment) **Enabled as survey** Active languages - Data Entry: vi | Survey: vi			
1	[record_id]	Record ID	text
2	[field_id]	Field ID:	text
3	[date]	Date:	text
4	[year_of_birth]	Year of Birth:	text
5	[major_specialty]	Major Specialty:	radio 1 Infectious Diseases 2 General Internal Medicine 3 Other (please specify)
6	[please_specify] Show the field ONLY if: [major_specialty] = '3'	Please specify:	text
7	[time_since_hepatitis_train]	Time Since Hepatitis Training:	radio 1 Less than 6 months 2 6 months to 1 year 3 More than 1 year
8	[involvement_in_hepatitis_s]	Involvement in Hepatitis Service:	checkbox 1 involvement_in_hepatitis_s___1 Diagnos and Treatme 2 involvement_in_hepatitis_s___2 Counsel and Screenir 3 involvement_in_hepatitis_s___3 None of the abo

9	[how_long_have_you_been_pra] Show the field ONLY if: [involvement_in_hepatitis_s] = "1" or "2"	How long have you been practicing hepatitis-related services?	radio 1 Less than 6 months 2 6 months to 1 year 3 More than 1 year
10	[on_average_how_many_hepati] Show the field ONLY if: [involvement_in_hepatitis_s] = "1"	On average, how many hepatitis patients do you see per month?	radio 1 Less than 10 2 10-25 3 More than 25
11	[mr_d_64_years_old_came_to]	Case 1: Mr. D, 64 years old, came to the clinic because of fatigue and loss of appetite for one month; yellowish eyes and dark urine in the past 3 weeks. The patient did not have fever, and had not taken any medicine at home. He has been seen by a primary care doctor, and got HBsAg+ from the testing. He was referred to your facility. The patient never complained of any significant jaundice and fatigue before. No family members were diagnosed with liver cirrhosis or HCC. Neither his wife nor children have hepatitis B/C. The patient has occasionally drunk alcohol for the past 20 years, 3 glasses of wine each time, and does not smoke. He tested positive for HBsAg around 5 years ago but never got viral load tested or treatment with antivirals as the test is not available at the primary care and he overall felt well.	descriptive
12	[question_1_choose_the_corr]	Question 1: Choose the correct answer from the following statements on risk of HBV infection and its treatment?	radio 1 A. Patients without liver-related symptoms have a low risk of progressing to the late stage of liver disease. 2 B. Traditional medicine can help the liver to control the replication of the virus. 3 C. Early diagnosis and appropriate management can prevent the progression to late stage liver diseases and death. 5 D. Only people with liver-related symptoms are eligible for antiviral treatment
13	[question_2_choose_the_best]	Question 2: Choose the best answer that describes the need for HBV viral load in a person newly diagnosed with HBV infection?	radio 1 A. The HBV viral load is only ordered when a person can pay for the test out of pocket 2 B. Patients without hepatic symptoms should be monitored with

			AST and ALT only but no HBV viral load 3 C. Patients with liver cirrhosis or liver cancer need to have HBV viral load tested every month. 4 D. All persons diagnosed with HBV infection should undergo a full evaluation, including HBV viral load.
14	[on_clinical_examination_yo]	On clinical examination, you found that his vital signs were normal. General condition: Patient is alert, is 175 tall, weighs 60 kg. Head and neck: normal. Skin and mucous membranes: yellow eyes, skin normal, no oedema. Abdominal examination: normal abdomen, no dullness to percussion in the lower area, liver was palpable at costal margins, and spleen not palpable. No other significant findings at clinical examinations.	descriptive
15	[lab_results]	The results of his lab tests and imaging investigations as as follows:	descriptive (Attachment: A1.png, Display format: Inline image/PDF)
16	[question_3_based_on_clinic]	Question 3: Based on clinical and laboratory findings above, fill in the table below on the diagnosis of disease.	descriptive
17	[interpretation_of_liver_en]	Interpretation of liver enzymes: (Provided that pt's ALT is 64 U/L)	radio 1 ALT is within ULN 2 ALT is elevated ≤ 2x ULN 3 ALT elevated > 2x ULN
18	[ast_within_uln_elevated_2x]	Interpretation of liver enzymes: (Provided that pt's AST - 79 U/L)	radio 1 AST within ULN 2 AST elevated ≤ 2x ULN 3 AST elevated > 2x ULN
19	[apri_score]	APRI score: _____ (Use https://nohepb.org/vietnamese/APRI-E.html for calculation; provided that pt's AST is 79 U/L, platelet - 98 G/L)	text
20	[apri_score_interpretation]	APRI Score Interpretation:	radio 1 F0-F1 2 F2 3 F3 4 F4
21	[child_pugh_score_use_https]	Child-Pugh score: _____ (Use https://www.msdmanuals.com/vi/professional/multimedia/table/thang-%C4%91i%E1%BB%83m-child-turcotte-pugh for calculation; provided that pt is in conscious, total bilirubin - 33.2 µmol/L, albumin 31 g/L, INR - 1.67, no ascitis on ultrasound)	text

22	[creatinine_clearance_ml_mi]	Creatinine clearance: _____ mL/min (Use https://hscc.vn/tools.asp?id=bantin&idnhom=6&idtin=111#google_vignette for calculation; provided that pt is 64 y.o., male, height 168 cm, weight 64 kg, creatinine - 77 µmol/L)	text
23	[co_infection_with_hcv_yes]	Co-infection with HCV:	yesno 1, Yes 0, No
24	[co_infection_with_hiv_yes]	Co-infection with HIV:	yesno 1, Yes 0, No
25	[hepatocellular_carcinoma_p]	Hepatocellular carcinoma present:	yesno 1, Yes 0, No
26	[specify_the_status_of_hbv]	Specify the status of HBV infection:	radio 1, Acute 2, Chronic
27	[if_chronic_no_to_mild_fibr] Show the field ONLY if: [specify_the_status_of_hbv] = "2"	If chronic:	radio 1, No to mild fibrosis 2, Significant fibrosis 3, Advanced fibrosis 4, Liver cirrhosis
28	[if_cirrhosis_compensated_d] Show the field ONLY if: [if_chronic_no_to_mild_fibr] = "4"	If cirrhosis:	radio 1, Compensated 2, Decompensated
29	[question_4_fill_in_the_tre]	Question 4: Fill in the treatment plan for the patient.	descriptive
30	[fill_in_the_management_pla]	Fill in the management plan for the patient.	descriptive
31	[need_to_be_referred_to_hig]	Need to be referred to higher level for management:	radio 1, Yes 2, No
32	[eligibility_for_hbv_antivi]	Eligibility for HBV antiviral:	radio 1, Yes 2, No
33	[if_yes_choice_of_antiviral] Show the field ONLY if: [eligibility_for_hbv_antivi] = "1"	If Yes, choice of antiviral:	radio 1, Adefovir 10 mg/day PO 2, Tenofovir disoproxil Fumarate (TDF) 300 mg/day PO

			3 Lamivudine 150 mg/day PO
34	[duration_of_treatment_unti] Show the field ONLY if: [eligibility_for_hbv_antivi] = "1"	Duration of treatment:	radio 1 Until ALT normalized 2 Until HBV DNA undetectable 3 Lifelong 4 Not applicable
35	[fill_in_the_monitoring_pla]	Question 5: Fill in the monitoring plan for the patient in the first year of HBV care:	descriptive
36	[month_three_from_the_start]	Month three from the start of antiviral	checkbox 1 month_three_from_the_start___1 Physic exam 2 month_three_from_the_start___2 Liver functi (at lea AST, A 3 month_three_from_the_start___3 Liver ultras and A 4 month_three_from_the_start___4 Creati creatin cleara 5 month_three_from_the_start___5 APRI s calcul 6 month_three_from_the_start___6 HBV vi load
37	[month_6_from_the_start_of]	Month 6 from the start of antiviral:	checkbox 1 month_6_from_the_start_of___1 Physica exam 2 month_6_from_the_start_of___2 Liver functio (at leas AST, AL 3 month_6_from_the_start_of___3 Liver ultraso and AF 4 month_6_from_the_start_of___4 Creatin creatini clearan 5 month_6_from_the_start_of___5 APRI sc calculat 6 month_6_from_the_start_of___6 HBV vir load
38	[month_nine_from_the_start]	Month nine from the start of antiviral:	checkbox 1 month_nine_from_the_start___1 Physica exam

			2 month_nine_from_the_start___2 Liver functio (at leas AST, AL 3 month_nine_from_the_start___3 Liver ultraso and AF 4 month_nine_from_the_start___4 Creatin creatin clearan 5 month_nine_from_the_start___5 APRI sc calcula 6 month_nine_from_the_start___6 HBV vir load
39	[month_12_from_the_start_of]	Month 12 from the start of antiviral:	checkbox 1 month_12_from_the_start_of___1 Physic exam 2 month_12_from_the_start_of___2 Liver functi (at lea AST, A 3 month_12_from_the_start_of___3 Liver ultras and A 4 month_12_from_the_start_of___4 Creati creati clearа 5 month_12_from_the_start_of___5 APRI s calcul 6 month_12_from_the_start_of___6 HBV v load
40	[choose_the_most_appropriat]	Question 6: Choose the most appropriate answer on counseling Mr. D on his condition and management?	radio 1 A. As he has recently developed symptoms, his hepatitis is in early stage and he should not be worried about that 2 B. Health insurance does not cover for the HBV viral load test that is done at other facility 3 C. The lifelong antiviral with good adherence helps liver to be healthy, much decrease the risk of advanced liver disease 4 D. Alcohol does no harm to the liver once he is on treatment with antiviral
41	[mrs_v_31_years_old_24_week]	Case 2: Mrs. V, 31 years old 24-week pregnant woman came to your health facility after being referred by a primary care physician because of her history of	descriptive

		HBsAg (+) for the last few years. No one has cirrhosis and HCC in Mrs. V's family. Her husband has never been tested with Hepatitis B or other liver-related illness. This is her second pregnancy, and the first children never got tested for Hepatitis B. Mrs. V has no underlying diseases and is not taking any medications. She does not experience fatigue, loss of appetite, yellow eyes, or any bleeding. No liver enzymes and virus count tests were done, so no medications were taken. HIV and syphilis are negative.	
42	[question_1_choose_the_most]	Question 1: Choose the most appropriate statement on risk the woman will transmit HBV virus to her baby?	radio 1 A. Very low to no risk as the woman does not have signs and symptoms of hepatitis when pregnant 2 B. Very low to no risk as the woman does not have HIV or syphilis co-infections 3 C. The risk can be as high as > 90% if her HBV DNA is > 200,000 IU/mL and/or HBeAg+ 4 D. No risk if the baby will get all three doses of HBV vaccine as part of expanded program of immunization
43	[question_2_what_should_be]	Question 2: What should be done for this woman?	radio 1 A. Nothing should be done to a HBV infected pregnant woman to keep her pregnancy and her baby safe 2 B. HBV DNA solely to estimate the risk of HBV transmission to her baby 3 C. Evaluation for liver function and HBV DNA thereafter if the ALT > 2x of ULN 4 D. Evaluation for both liver disease and HBV activity
44	[on_clinical_examination_th]	On clinical examination, the woman's vital signs are normal. She is alert and responsive; 158 cm tall, weighs 65 kg. Head and neck: normal. Skin: normal skin and mucous membranes. No edema, no subcutaneous hemorrhage. Abdominal examination: the liver and spleen were not palpable. Cardiac, pulmonary, neurological, and OnG examination: normal.	descriptive
45	[the_results_of_laboratory]	The results of laboratory analyses and other investigations:	descriptive (Attachment: A2.png, Display format: Inline image/PDF)
46	[question_3_based_on_clinic1]	Question 3: Based on clinical and laboratory findings above, fill in the table below on the diagnosis of disease.	descriptive

47	[interpretation_of_liver_en_2]	Question 3a: Interpretation of liver enzymes: (Provided that pt's ALT is 20 U/L)	radio 1 ALT is within ULN 2 ALT is elevated ≤ 2x ULN 3 ALT elevated > 2x ULN
48	[ast_within_uln_elevated_2x_2]	Question 3a: Interpretation of liver enzymes: (Provided that pt's AST - 19 U/L)	radio 1 AST within ULN 2 AST elevated ≤ 2x ULN 3 AST elevated > 2x ULN
49	[apri_score_2]	Question 3b: APRI score: _____ (Use https://nohepb.org/vietnamese/APRI-E.html for calculation; provided that pt's AST is 19 U/L, platelet - 250 G/L)	text
50	[sdfdsf]	APRI Score Interpretation:	radio 1 F0-F1 2 F2 3 F3 4 F4
51	[co_infection_with_hcv_yes_2]	Question 3c: Co-infection with HCV:	yesno 1 Yes 0 No
52	[co_infection_with_hiv_yes_2]	Question 3c: Co-infection with HIV:	yesno 1 Yes 0 No
53	[hepatocellular_carcinoma_p_2]	Question 3d: Hepatocellular carcinoma present:	yesno 1 Yes 0 No
54	[specify_the_status_of_hbv_2]	Question 3e: Specify the status of HBV infection:	radio 1 Acute 2 Chronic
55	[if_chronic_no_to_mild_fibr_2] Show the field ONLY if: [specify_the_status_of_hbv_2] = "2"	If chronic:	radio 1 No to mild fibrosis 2 Significant fibrosis 3 Advanced fibrosis 4 Liver cirrhosis
56	[if_cirrhosis_compensated_d_2] Show the field ONLY if: [if_chronic_no_to_mild_fibr_2] = "4"	If cirrhosis:	radio 1 Compensated 2 Decompensated 3 Not Relevant
57	[fill_in_the_treatment_plan]	Question 4: Fill in the treatment plan for the patient.	descriptive

58	[need_to_be_referred_to_hig1]	Question 4a: Need to be referred to higher level for management:	yesno 1 Yes 0 No
59	[eligibility_for_hbv_antivi1]	Question 4b: Eligibility for HBV antiviral for the woman's own health:	yesno 1 Yes 0 No
60	[if_yes_choice_of_antiviral1] Show the field ONLY if: [eligibility_for_hbv_antivi1] = "1"	If Yes, choice of antiviral:	radio 1 Entecavir 0.5 mg/day PO 2 Lamivudine 150 mg/day PO 3 Tenofovir disoproxil Fumarate (TDF) 300 mg/day 4 Not applicable
61	[need_of_antiviral_for_prev]	Question 4c: Need of antiviral for preventing HBV transmission to baby:	yesno 1 Yes 0 No
62	[if_yes_choice_of_antiviral2] Show the field ONLY if: [need_of_antiviral_for_prev] = "1"	If Yes, choice of antiviral:	radio 1 Entecavir 0.5 mg/day PO 2 Lamivudine 150 mg/day PO 3 Tenofovir disoproxil Fumarate (TDF) 300 mg/day 4 Not applicable
63	[question_5_which_statement]	Question 5: Which statement of the following is correct on the preventing mother-to-child transmission of HBV infection in this case?	radio 1 A. The baby should receive prophylaxis with oral lamivudine after birth 2 B. The mother should receive peripartum TDF until a week after delivery 3 C. The baby should receive HBV-IG, first dose of HBV vaccine within 24hr after birth and all HBV doses following EPI 4 D. Give HBV IG to the baby if he/she tested positive for HBsAg after birth
64	[question_6_which_statement]	Question 6: Which statement of the following is correct on the plan of monitoring the woman's and the baby's HBV status?	radio 1 A. Re-assess the woman only if she develops signs and symptoms of hepatitis 2 B. Do clinical workup and HBV viral load for the woman 3 months after delivery or sooner if she develops hepatitis symptoms 3 C. Test HBsAg for the baby at 6 months, right after receiving the last

			dose of HBV vaccine 4 D. Do nothing for the woman and the baby as they are safe with the management plan
65	[question_7_of_the_following]	Question 7: Of the following statements, which one is correct on how to counsel the woman on her condition and management?	radio 1 A. She does not need antiviral for her HBV infection or her baby but needs to follow baby preventive measures 2 B. She can have traditional medicine to save her baby from infection 3 C. She should not breast feed her baby to prevent the transmission of HBV 4 D. Her first baby and her husband do not need HBsAg test as they both have no symptoms of hepatitis
66	[case_3_mr_m_34_years_old_m]	CASE 3: Mr. M, 34 years old, male, has been referred to you as after having tested HBsAg positive at a health check. He is otherwise in good health, never uses injection drugs or smokes; drinks 1-2 times/month with 2-3 glasses of beer each time. No family members ever had liver diseases before and not tested for viral hepatitis. He is already married and has two children, and all his wife and the children have never been tested for Hepatitis B.	descriptive
67	[question_1_the_following_s]	Question 1: The following statements are about a person tested positive with an HBsAg screening test, which one is CORRECT?	radio 1 A. The person is immune to HBV 2 B. The person inherits the HBV genetically from his parents 3 C. The person was previously vaccinated against HBV 4 D. The person is infected with HBV
68	[question_2_the_following_s]	Question 2: The following statements are about the eligibility for antiviral treatment for a HBV infected persons, which one is CORRECT?	radio 1 A. All HBV infected people are eligible for antiviral treatment 2 B. Only persons with HBV DNA level of > 200,000 IU/mL need treatment with antiviral 3 C. All persons with liver cirrhosis need treatment with antiviral at any positive HBV DNA levels 4 D. Only persons with liver cancer should be given antiviral treatment
69	[on_a_clinical_exam_the_pat]	On a clinical exam, the patient's vital signs were normal. He was alert and responsive. Head and neck: normal; skin and mucous - normal, no jaundice, edema, or subcutaneous hemorrhage.	descriptive

		Abdominal examination: abdomen not distended; liver and spleen not palpable. Cardiac, pulmonary, and neurological examination was normal.	
70	[case3_labresults]	The liver test and other studies' results as below:	descriptive (Attachment: A3.png, Display format: Inline image/PDF)
71	[question_3_based_on_clinic5]	Question 3: Based on clinical and laboratory findings above, fill in the table below on the diagnosis of disease.	descriptive
72	[interpretation_of_liver_en_3]	Question 3a: Interpretation of liver enzymes: (Provided that pt's ALT is 39 U/L)	radio 1 ALT is within ULN 2 ALT is elevated ≤ 2x ULN 3 ALT elevated > 2x ULN
73	[ast_within_uln_elevated_2x_3]	Question 3a: Interpretation of liver enzymes: (Provided that pt's AST - 36 U/L)	radio 1 AST within ULN 2 AST elevated ≤ 2x ULN 3 AST elevated > 2x ULN
74	[apri_score_3]	Question 3b: APRI score: _____ (Use https://nohepb.org/vietnamese/APRI-E.html for calculation; provided that pt's AST is 36 U/L, platelet - 280 G/L)	text
75	[apri_score_interpretation3]	APRI Score Interpretation:	radio 1 F0-F1 2 F2 3 F3 4 F4
76	[co_infection_with_hcv]	Question 3c: Co-Infection with HCV	yesno 1 Yes 0 No
77	[co_infection_with_hiv]	Question 3c: Co-Infection with HIV	yesno 1 Yes 0 No
78	[hepatocellular_carcinoma_p1]	Question 3d: Hepatocellular carcinoma present:	yesno 1 Yes 0 No
79	[specify_the_status_of_hbv_3]	Question 3e: Specify the status of HBV infection:	radio 1 Acute 2 Chronic

80	[if_chronic_no_to_mild_fibr_3] Show the field ONLY if: [specify_the_status_of_hbv_3] = "2"	If chronic:	radio 1 No to mild fibrosis 2 Significant fibrosis 3 Advanced fibrosis 4 Liver cirrhosis
81	[if_cirrhosis_compensated_d_3] Show the field ONLY if: [if_chronic_no_to_mild_fibr_3] = "4"	If cirrhosis:	radio 1 Compensated 2 Decompensated 3 Not Relevant
82	[fill_in_the_treatment_plan1]	Question 4: Fill in the treatment plan for the patient:	descriptive
83	[need_to_be_referred_to_hig2]	Question 4a: Need to be referred to higher level for management:	yesno 1 Yes 0 No
84	[eligibility_for_hbv_antivi2]	Question 4b: Eligibility for HBV antiviral:	yesno 1 Yes 0 No
85	[if_yes_choice_of_antiviral3] Show the field ONLY if: [eligibility_for_hbv_antivi2] = "1"	If Yes, choice of antiviral:	radio 1 Entecavir 0.5 mg/day PO 2 Lamivudine 150 mg/day PO 3 Tenofovir disoproxil Fumarate (TDF) 300 mg/day 4 Not applicable
86	[question_5_what_is_the_fol]	Question 5: What is the follow-up plan for this patient (choose most appropriate answer)?	radio 1 A. He needs to be monitored every month 2 B. He should visit hepatitis clinic 3 - 6 months 3 C. He needs to come back when found to have liver cirrhosis 4 D. He is well, no need for follow-up visits
87	[choose_the_most_appropriat1]	Question 6: Choose the most appropriate answer on counseling Mr. M on his condition and management? A. He currently does not need antiviral medication but traditional medicine can help liver to fight the virus B. He is in good liver health then his family members (parents, siblings, wife and children) can be safe without testing for their HBV status C. He needs to follow the scheduled monitoring for his HBV disease progression for timely antiviral treatment when eligible	radio 1 A. He currently does not need antiviral medication but traditional medicine can help liver to fight the virus 2 B. He is in good liver health then his family members (parents, siblings, wife and children) can be safe without testing for their HBV status 3 C. He needs to follow the scheduled monitoring for his HBV disease

		D. In case he is worried about his condition, he can buy and take the antiviral medicine on his own until feeling well.	progression for timely antiviral treatment when eligible 4 D. In case he is worried about his condition, he can buy and take the antiviral medicine on his own until feeling well.
88	[question_7_please_interpre]	Question 7: Please, interpret the status of HBV infection of Mr. M. family members.	descriptive
89	[case3_testresults]		descriptive (Attachment: A4.png, Display format: Inline image/PDF)
90	[wife]	7a: Wife:	radio 1 HBV infected 2 Resolved infection 3 Not infected, immune 4 Not infected, nonimmune (needs vaccine)
91	[child_1]	7b: Child #1:	radio 1 HBV infected 2 Resolved infection 3 Not infected, immune 4 Not infected, nonimmune (needs vaccine)
92	[child_2]	7c: Child #2:	radio 1 HBV infected 2 Resolved infection 3 Not infected, immune 4 Not infected, nonimmune (needs vaccine)
93	[mr_m_also_shares_that_his]	Mr. M. also shares that his parents and his two siblings were also tested for HBV, with his mother and one brother turned out to be infected with HBV; her mother is diagnosed with compensated liver cirrhosis and is on antiviral medication.	descriptive
94	[question_8_with_that_turn]	Question 8: With that turn of Mr. M's family history, what would you do?	radio 1 A. Do nothing, wait for Mr. M. next clinical visit scheduled at month 6 2 B. Arrange early assessment to save him from month 6 visit 3 C. Order a CT scan to see if he has liver cirrhosis like his mother 4 D. Prescribe antiviral and counsel on treatment adherence.
95	[case_4_mr_p_28_years_old_a]	Case 4: Mr. P., 28 years old, attended your hospital	descriptive

		because of experiencing loss of appetite, nausea, and weakness for the past 3 weeks. He had no fever, no shortness of breath, no jaundice but a mild dark colour of urine. As he felt uncomfortable, he sought traditional medicine but did not get better. Mr. P. was tested positive for anti-HCV two years ago when he volunteered for blood donation, but has never got further investigation for this condition. He was overall well, except sometimes having dark red rash around the knees. Mr. P is married with a wife and a child, and they are healthy and never got tested for Hepatitis C. Medical history: Mr. P. received a red cell transfusion 7 years ago because of his motorbike accident. He hasn't been vaccinated against Hepatitis B or taken any drugs or alcohol, and does not smoke. No HCC and liver cirrhosis in his family.	
96	[question_1_choose_the_corr1]	Question 1: Choose the correct answer from the following statements on the HCV infection.	radio 1 A. People infected with HCV have high risk of progression to chronic hepatitis, liver cirrhosis and other end-stage liver diseases 2 B. All people who get HCV infection will invariously become chronically infected after 6 months' time 3 C. Anti-HCV is a definitive diagnosis of chronic HCV infection 4 D. Co-infection with HIV makes no difference in the course of HCV infection compared to HCV mono-infection
97	[question_2_what_is_the_mea]	Question 2: What is the meaning and the use of HCV viral load test for a person with a positive anti-HCV test (choose the most appropriate answer)?	radio 1 A. HCV viral load is only reserved for HIV co-infected people 2 B. High HCV viral load > 106 IU/mL is indicative of acute HCV infection 3 C. Only ordered when people have evidence of liver cirrhosis 4 D. To confirm if the HCV infection is viremic and needs antiviral treatment
98	[at_the_clinical_examinatio]	At the clinical examination, his vital signs were normal; head and neck: normal; no jaundice or subcutaneous hemorrhage; some palpable purpura on both of his legs. Abdominal examination: the abdomen is soft, not distended; liver and spleen not palpable. The other physical examinations are normal.	descriptive
99	[case4labresults]	His laboratory analyses and investigation results presented below:	descriptive (Attachment: A5.png, Display format:

			Inline image/PDF)
100	[question_3_based_on_clinic2]	Question 3: Based on clinical and laboratory findings above, fill in the table below on the diagnosis of disease.	descriptive
101	[interpretation_of_liver_en_4]	Question 3a: Interpretation of liver enzymes: (Provided that pt's ALT is 40 U/L)	radio 1 ALT is within ULN 2 ALT is elevated ≤ 2x ULN 3 ALT elevated > 2x ULN
102	[ast_within_uln_elevated_2x_4]	Question 3a: Interpretation of liver enzymes: (Provided that pt's AST - 42 U/L)	radio 1 AST within ULN 2 AST elevated ≤ 2x ULN 3 AST elevated > 2x ULN
103	[apri_score_4]	Question 3b: APRI score: _____ (Use https://nohepb.org/vietnamese/APRI-E.html for calculation; provided that pt's AST is 64 U/L, platelet - 110 G/L)	text
104	[sdfsdfdsf]	APRI Score Interpretation:	radio 1 F0-F1 2 F2 3 F3 4 F4
105	[co_infection_with_hbv]	Question 3c: Co-infection with HBV:	radio 1 Yes 2 No 3 Recovered
106	[co_infection_with_hiv1]	Question 3c: Co-infection with HIV:	yesno 1 Yes 0 No
107	[most_appropriate_explanati]	Question 3d: Most appropriate explanation for the palpable purpura on both of his legs?	radio 1 Hypothrombocytopenic purpura 2 Cryoglobulinemia related vasculitis 3 Altered synthesis of coagulation factors in the liver
108	[hepatocellular_carcinoma_p2]	Question 3e: Hepatocellular carcinoma present:	yesno 1 Yes 0 No
109	[specify_the_status_of_hcv]	Question 3f: Specify the status of HCV infection:	radio 1 Viremic 2 Resolved
110	[fibrotic_stage]	Fibrotic stage	radio 1 No to mild fibrosis

			2 Significant fibrosis 3 Advanced fibrosis 4 Liver cirrhosis
111	[if_cirrhosis] Show the field ONLY if: [fibrotic_stage] = "4"	If cirrhosis:	radio 1 Compensated 2 Decompensated 3 Not Relevant
112	[fill_in_the_treatment_plan2]	Question 4: Fill in the treatment plan for the patient:	descriptive
113	[need_to_be_referred_to_hig3]	Question 4a: Need to be referred to higher level for management:	yesno 1 Yes 0 No
114	[eligibility_for_anti_hcv_a]	Question 4b: Eligibility for anti-HCV antivirals (DAAs):	yesno 1 Yes 0 No
115	[if_yes_choice_of_daas] Show the field ONLY if: [eligibility_for_anti_hcv_a] = "1"	If Yes, choice of DAAs:	radio 1 SOF/VEL or SOF/DCV QD x 12 wks 2 SOF/VEL or SOF/DCV QD x 24 wks 3 Needs HCV genotyping before either of DDA regimen above
116	[the_following_are_messages]	Question 5: The following are messages on the treatment with directly acting antivirals (DAAs) for HCV infection, which one is CORRECT?	radio 1 A. The DAAs are effective in clearing the virus 2 B. In patients with extrahepatic manifestations of HCV infection, treatment with DAA can only be started after those manifestations are well controlled 3 C. The treatment with DAA will last at least one year to be effective 4 D. Treatment with DAAs eliminate the risk of liver related death even in patients with advanced liver fibrosis or cirrhosis
117	[the_patient_completed_the]	The patient completed the DAA regimen per prescription and returned for re-evaluation at week 15 after completion of DAAs. He feels good, the appetite returned; overall looks well; no more jaundice; the purpura on the legs are much subsided.	descriptive
118	[case4_testresults]	His laboratory analyses and investigation results presented below:	descriptive (Attachment: A6.png, Display format: Inline image/PDF)
119	[apri_score_5]	Question 6a: APRI score: _____ (Use https://nohepb.org/vietnamese/APRI-E.html	text

		for calculation; provided that pt's AST is 59 U/L, platelet - 112 G/L)	
120	[apri_score_interpretation2]	APRI Score Interpretation:	radio 1 F0-F1 2 F2 3 F3 4 F4
121	[question_6_state_the_hcv_a1]	Question 6: State the HCV and fibrosis staging and the status of HCV infection of this patient?	descriptive
122	[status_of_hcv_infection]	Question 6a: Status of HCV infection:	radio 1 Achieving SVR 12 (cured) 2 Not cured
123	[question_7_based_on_hcv_rn]	Question 7: Based on HCV RNA result, the patient is classified as cured for HCV infection, advanced fibrosis stage. Which statement is correct on counseling the patient after the SVR result?	radio 1 A. He is free from the HCV now (cure) but not immune to re-infection if exposed in the future 2 B. He needs another HCV viral load test after 12 weeks to confirm cure 3 C. He has no risk of progression to liver cirrhosis from now on and does not need further monitoring 4 D. In case of re-exposure to HCV, he needs to take another anti-HCV test to diagnose his re-infection
124	[case_5_mr_h_30_years_old_w]	Case 5: Mr. H, 30 years old, was diagnosed with HIV in November 2017 and on ARV treatment TDF/3TC/EFV regimen. When he was enrolled, he was tested and got anti-HCV (+) and HBsAg (+). At that time, he was not offered to get HCV RNA test, or HCV antiviral treatment. He has a history of using IV drugs when he was young, but has stopped for the past 7 years. Mr. H. developed yellow eyes for the last 2 months, also experienced fatigue and loss of appetite, and light discoloration of stool at the same time. He had no fever, no nausea or vomiting. His mother and his sister are diagnosed with HBV-infection but no one in his family was diagnosed with liver cancer or liver cirrhosis.	descriptive
125	[question_1_choose_the_corr2]	Question 1: Choose the correct answer from the following statements on the HCV and HBV co-infection in people living with HIV (PLHIV).	radio 1 A. PLHIV need to be tested for HBV and HCV co-infections only if having been engaged in IV drug use or have signs and symptoms of hepatitis 2 B. The anti-HCV test is mostly negative among PLHIV with HCV due to HIV related immunodeficiency

			3 C. HBsAg is usually negative in HBV infected PLHIV due to HIV related immunodeficiency 4 D. Co-infection with HIV makes HCV and/or HBV infection more complicated compared to mono-infection
126	[question_2_choose_the_corra_hiv_treatment_with_arv_should_be_discontinued_when_hbv_hcv_co_infected]	Question 2: Choose the correct answer from the following statements on the management of HCV and HBV co-infection in PLHIV?	radio 1 A. HIV treatment with ARV should be discontinued when HBV/HCV co-infected PLHIV are on HBV/HCV antivirals 2 B. The cure rate for HCV infection when treated with DAAs is almost the same for people co-infected with HIV and people with HCV mono-infection 3 C. PLHIV with HBV/HCV co-infection should be referred to provincial or central levels for management 4 D. PLHIV with HBV and/or HCV co-infection need different HBV or HCV antiviral regimens rather than the ones that are used for mono-infected people.
127	[on_clinical_examination_mr]	On clinical examination, Mr. H. is generally well, alert and responsive. He is 170cm tall, weighs 64 kg. His skin and conjunctiva are light yellow; no edema or subcutaneous hemorrhage; abdomen is soft, liver and spleen not palpable. Cardiac, pulmonary, and neurological examination is normal.	descriptive
128	[case5labresults]	The results of Mr. H's laboratory and imaging investigations are in the table below:	descriptive (Attachment: A7.png, Display format: Inline image/PDF)
129	[question_3_based_on_clinic3]	Question 3: Based on clinical and laboratory findings above, fill in the table below on the diagnosis of disease.	descriptive
130	[interpretation_of_liver_en_5]	Question 3a: Interpretation of liver enzymes: (Provided that pt's ALT is 70 U/L)	radio 1 ALT is within ULN 2 ALT is elevated ≤ 2x ULN 3 ALT elevated > 2x ULN
131	[ast_within_uln_elevated_2x_5]	Question 3a: Interpretation of liver enzymes: (Provided that pt's AST - 100 U/L)	radio 1 AST within ULN 2 AST elevated ≤ 2x ULN 3 AST elevated > 2x ULN
132	[apri_score_6]	Question 3b: APRI score: _____ (Use https://nohepb.org/vietnamese/APRI-E.html	text

		for calculation; provided that pt's AST is 100 U/L, platelet - 115 G/L)	
133	[apri_score_interpretation1]	APRI Score Interpretation:	radio 1 F0-F1 2 F2 3 F3 4 F4
134	[child_pugh_score_use_https_2]	Question 3c: Child-Pugh score: _____ (Use https://www.msdmanuals.com/vi/professional/multimedia/table/thang-%C4%91i%E1%BB%83m-child-turcotte-pugh for calculation; provided that pt is in conscious, total bilirubin - 38 µmol/L, albumin 31 g/L, INR - 1.62, no ascitis on ultrasound)	text
135	[hepatocellular_carcinoma_p3]	Question 3d: Hepatocellular carcinoma present:	yesno 1 Yes 0 No
136	[status_of_hbv_infection]	Question 3e: Status of HBV infection:	radio 1 Not treated, not virally suppressed 2 Treated, virally suppressed 3 Resolved
137	[status_of_hcv_infection1]	Question 3f: Status of HCV infection:	radio 1 Viremic 2 Resolved
138	[fibrotic_stage_of_liver]	Question 3g: Fibrotic stage of liver	radio 1 No to mild fibrosis 2 Significant fibrosis 3 Advanced fibrosis 4 Liver cirrhosis
139	[if_cirrhosis1] Show the field ONLY if: [fibrotic_stage_of_liver] = "4"	If cirrhosis:	radio 1 Compensated 2 Decompensated 3 Not Relevant
140	[status_of_hiv_infection]	Question 3h: Status of HIV infection	radio 1 Not treated 2 On ART, virally not suppressed 3 On ART, virally suppressed
141	[fill_in_the_treatment_plan3]	Question 4: Fill in the treatment plan for the patient:	descriptive
142	[eligibility_for_anti_hcv_a1]	Question 4a: Eligibility for anti-HCV antivirals (DAAs):	yesno 1 Yes

			0 | No
143	[if_yes_choice_of_daas1] Show the field ONLY if: [eligibility_for_anti_hcv_a1] = "1"	If Yes, choice of DAAs:	radio 1 | SOF/VEL or SOF/DCV QD x 12 wks 2 | SOF/VEL or SOF/DCV QD x 24 wks 3 | Needs HCV genotyping before either of DDA regimen above
144	[the_antiviral_regimen_that]	The antiviral regimen that is available and free for HIV patients in the facility is SOF 400 mg + DCV 60 mg; the patient's ARV regimen had been switched to TDF/3TC/DTG to avoid the drug-drug interaction between EFV and DCV. The patient was counseled on taking the DAAs and ARVs as prescribed, avoiding alcohol, and other healthy living measures. Mr. H. came back to the clinic after 4 weeks of treatment. He reported good compliance with the medicines, not missing any dose of both ARVs and DAAs; the nausea subsided and he gained appetite; the discoloration of skin and eyes had gone; and the patient gained some weight. On clinical examination, no specific signs and symptoms were found.	descriptive
145	[case5results]	As you ordered some tests to evaluate his condition, you got result:	descriptive (Attachment: A8.png, Display format: Inline image/PDF)
146	[question_5_the_following_s]	Question 5: The following statements are what to advise the patient on his condition and the treatment, please choose the one that is correct.	radio 1 | A. He has good response to anti-HCV regimen and the treatment time with DAAs can be shortened to 12 weeks instead of 24 weeks 2 | B. He needs to have a second HCV RNA test now to confirm the response to the DAA regimen 3 | C. The hepatitis status is much improved and he can enjoy some alcohol drink from now 4 | D. The treatment plan stays the same and he needs to maintain good adherence to it
147	[mr_h_completed_his_24_week]	Mr. H. completed his 24-weeks DAA regimen with good compliance and came back to the clinic 14 weeks after completion of the DAAs. He is currently very healthy and still continuing his ART with TDF/3TC/DTG. From the clinical examination, Mr. H. is overall well; no hepatitis-related symptoms were found; liver and spleen were not palpable.	descriptive
148	[labresults5]	The results of his lab tests and imaging investigations as as follows:	descriptive (Attachment: A9.png, Display format: Inline image/PDF)

149	[question_6_state_the_hcv_a]	Question 6: State the HCV and fibrosis staging and the status of HCV infection of this patient?	descriptive
150	[apri_score_7]	Question 6a: APRI score: _____ (Use https://nohepb.org/vietnamese/APRI-E.html for calculation; provided that pt's AST is 100 U/L, platelet - 115 G/L)	text
151	[apri_score_interpretationssdfsd]	APRI Score Interpretation:	radio 1, F0-F1 2, F2 3, F3 4, F4
152	[child_pugh_score_use_https_3]	Question 6b: Child-Pugh score: _____ (Use https://www.msdmanuals.com/vi/professional/multimedia/table/thang-%C4%91i%E1%BB%83m-child-turcotte-pugh for calculation; provided that pt is in conscious, total bilirubin - 38 µmol/L, albumin 31 g/L, INR - 1.62, no ascitis on ultrasound)	text
153	[status_of_hcv_infection2]	Question 6c: Status of HCV infection:	radio 1, Achieving SVR 12 (cured) 2, Not cured
154	[fibrotic_stage_of_liver1]	Question 6d: Fibrotic stage of liver:	radio 1, No to mild fibrosis 2, Significant fibrosis 3, Advanced fibrosis 4, Liver cirrhosis
155	[if_cirrhosis2] Show the field ONLY if: [fibrotic_stage_of_liver1] = "4"	If cirrhosis:	radio 1, Compensated 2, Decompensated 3, Not Relevant
156	[question_7_what_is_appropr]	Question 7: What is appropriate to counsel the patient on his status after completion of HCV treatment?	radio 1, A. His HCV is cured and he has no risk of re-infection with the virus in the future 2, B. He needs to keep good adherence to his ART, which is also treating his HBV infection 3, C. As his HBV is well controlled and HCV is cured, the patient has no risk of liver cirrhosis or liver cancer from now on 4, D. His anti-HCV will become negative along with his HCV RNA
157	[hcp_competency_assessment_complete]	Section Header: *Form Status* Complete?	dropdown